

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 02-Apr-2026

1. Administrative & Study Identification

Full Study Title: Describing Treatment Patterns and Creating an Updated Treatment Flow in an Ulcerative Colitis Population **Short Title/Acronym:** EFFECT-1LAT **Protocol Number:** EFFECT-1LAT **NCT Number:** NCT07177209 **Sponsor Name:** Pfizer **Investigational Product:** Non-Interventional / Advanced Therapies (including etrasimod, ozanimod, upadacitinib, filgotinib, tofacitinib, vedolizumab, guselkumab, ustekinumab, risankizumab, adalimumab, infliximab, golimumab) **Phase of Development:** N/A (Observational/Registry Study) **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To map treatment use from conventional therapies to advanced therapies from the time of Ulcerative Colitis (UC) diagnosis in the Lothian Inflammatory Bowel Disease Registry (LIBDR). **Secondary Objectives:** To describe the effects of changing from conventional treatment to advanced treatments on the control of inflammation (C-Reactive Protein [CRP] or Faecal Calprotectin [Fcal]). To generate longitudinal Fcal profiles and inflammatory response models for patients on conventional therapy, and to determine the proportion of patients achieving early control of inflammation within 6-24 months of UC diagnosis.

2.2 Background & Rationale To address the gap in understanding real-world treatment sequencing and longitudinal inflammatory control in UC. The Lothian IBD registry (LIBDR) was compiled through a capture-recapture methodology to describe the observed prevalence of IBD. By analyzing approximately 4,000 UC patients (with ~3,200 on conventional therapy and >600 on advanced therapies), this study provides real-world evidence regarding the transition patterns and clinical outcomes associated with advancing from conventional to targeted biologic and small-molecule therapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Non-Interventional Registry Study) **Control Type:** N/A (Descriptive cohort design) **Blinding:** N/A (Open-label / Registry database extraction) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$\$\$: Approximately 4,000 patients **Number of Sites:** Single regional registry (Lothian

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Patient enrolled in the LIBDR IBD registry from 2009 onwards. 2. Age > 18 years at the time of diagnosis. 3. Confirmed diagnosis of Ulcerative Colitis (UC). 4. Patient data entered during the defined study period. 5. Minimum duration of data post-UC diagnosis (Cohort A: 6 months, Cohort B-D: 6 months).

4.2 Exclusion Criteria (Implicit based on registry parameters) 1. Diagnosis of Crohn's disease, indeterminate colitis, or other non-UC inflammatory bowel diseases. 2. Insufficient longitudinal follow-up data (< 6 months post-UC diagnosis).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: N/A (Observational extraction of real-world prescribing patterns) **Route of Administration:** N/A (Oral/IV/SC parameters collected as prescribed in standard practice) **Duration of Treatment:** Retrospective and prospective data spanning from 2008 up to August 1, 2025.

5.2 Concomitant & Prohibited Therapy Permitted: All standard-of-care conventional therapeutics and advanced therapies (e.g., TNF inhibitors, IL-12/23 inhibitors, JAK inhibitors, S1P modulators) utilized in routine clinical practice. **Prohibited:** N/A (Real-world observational cohort).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Index Date	Day 0 Time of UC Diagnosis / Registry Entry; Baseline demography
2	Early Assessment	Months 6-24 Evaluate early control of inflammation on conventional therapeutics (CRP/Fcal mapping)
3	Longitudinal Follow-up	2008 - Aug 2025 Map transition to advanced therapies; track longitudinal inflammatory response models
4	Data Extraction	Dec 2025 - Mar 2026 Final database lock and descriptive analysis

(Note: No physical visits are required as this is an observational registry data extraction)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Map of treatment flow and patterns from conventional therapies to advanced therapies calculated from the time of UC diagnosis up to August 1, 2025.

Measurement Method: Registry database analysis of prescription and treatment sequence records.

7.2 Secondary & Exploratory Endpoints * Change from baseline in control of inflammation (C-Reactive Protein [CRP] or Faecal Calprotectin [Fcal]) for patients transitioning to advanced treatments. * Generation of Fcal profile and longitudinal inflammatory response models for patients on conventional therapy. * Proportion of patients achieving early control of inflammation on conventional therapeutics within 6-24 months of UC diagnosis.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Evaluated retrospectively through registry documentation (e.g., reasons for treatment discontinuation, adverse drug reactions captured in electronic health records). **Serious Adverse Events (SAEs):** Standard pharmacovigilance reporting guidelines for observational data will apply. **Data Monitoring Committee (DMC):** Overseen by local registry governance (LIBDR data custodians) rather than an active trial DMC.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Registry Cohort (All UC patients matching inclusion criteria, N $\approx$ 4,000). **Sample Size**

Justification: Based on the observed prevalence of IBD in the Lothian region capturing ~4,000 UC patients, of which ~3,200 are on conventional therapy and >600 have commenced advanced therapy, providing sufficient power for descriptive statistics and modeling. **Handling of Missing Data:** Missing data will be handled according to registry epidemiological standards, likely utilizing multiple imputation for laboratory values (CRP/Fcal) where appropriate.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and local registry ethics and privacy approvals. **Monitoring Plan:** Centralized data quality checks and capture-recapture methodology validation by LIBDR administrators. **Audit Trail:** Secure database query logs and

reproducible extraction codes for treatment mapping and response modeling.

11. Study Identifiers & Governance

Primary ID: EFFECT-1LAT **Registry Number:** NCT07177209 **Sponsor/Lead Organization:** Pfizer **Study Title:** Describing Treatment Patterns and Creating an Updated Treatment Flow in an Ulcerative Colitis Population **Official Regulatory Title:** Describing Treatment Patterns and Creating an Updated Treatment Flow in an Ulcerative Colitis Population

12. Clinical Framework (Ulcerative Colitis Specific)

Primary Condition: Ulcerative Colitis (UC) **Diagnosis Threshold:** Clinically confirmed UC diagnosis **Medical Methodology:** Treatment transition mapping (Conventional to Advanced Therapies) **Optimization Target:** Control of mucosal inflammation (normalization of CRP and Faecal Calprotectin)

13. Study Procedures & Methodology

Management Pathway: Standard real-world clinical pathways for UC in the Lothian region. **Education Protocol (HCP):** N/A (Retrospective/Prospective observational data) **Education Protocol (Patient):** N/A (Registry tracking) **Quality Audit Mechanism:** Capture-recapture methodology for IBD prevalence and registry validation.

14. Observation & Follow-Up Schedule

Total Duration: Retrospective analysis covering 2008 to August 1, 2025 **Onsite Visit Cadence:** N/A (Real-world clinical visits dictated by standard of care) **Remote Monitoring Frequency:** Continuous registry data linkage **Checklist Review Interval:** Key outcome extraction at 6-24 months post-diagnosis and at times of therapy switch.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				